

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 72 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: April 20, 2017 | Provider: Priya Nair, MD — Endocrinology | Doc ID: RFH-2017-0420-ENDO

Endocrinology Consultation

Type 2 Diabetes — Multifactorial Optimization

Reason for Consultation

Co-management of T2DM with CKD G3a and rising A1c, referred by Dr. Park.

Assessment

72-year-old with 7-year history of T2DM, now A1c 8.2% on metformin + recently added empagliflozin. Microvascular complications: diabetic kidney disease (A3 albuminuria); retinal screening pending. Macrovascular: established CAD. Individualized A1c target 7.0-7.5% given age and comorbidity (avoid hypoglycemia).

Plan

- Continue metformin 1000 mg BID and empagliflozin 10 mg daily; up-titrate empagliflozin to 25 mg daily after 4 weeks if tolerated.
- If A1c remains >7.5% at 3 months, consider adding a GLP-1 receptor agonist (cardiorenal/weight benefit) over sulfonylurea.
- Reinforce diet, foot care, home glucose logging.
- Ensure annual dilated retinal exam and ACR monitoring.
- Statin and BP optimized — continue.

Follow-up

Endocrinology in 3-4 months with repeat A1c; coordinate with nephrology if eGFR declines.

Endocrine Recommendations

Medication	Dose	Route	Frequency	Indication
Empagliflozin	10 mg → 25 mg	PO	Once daily	Titrate up in 4 weeks
Metformin	1000 mg	PO	Twice daily	Continue